

Sexually Transmitted Infections (STIs) – Summary

Key Symptoms: Dysuria, discharge, lower abdominal pain, dyspareunia, bleeding, genital lesions, etc.

Most common causing-pathogens

- *N. gonorrhoeae*, *C. trachomatis*, HPV, *T. pallidum* (syphilis), *H. ducreyi*, *K. granulomatis*, etc.

Non-Pharmacological Treatment: Practice safe sex, post-coital urination

Pharmacological Treatment

- **Gonorrhea:** Ceftriaxone (1 dose, IM)
 - **Alternatives:** Cefixime + azithromycin **OR** cefixime + doxycycline **OR** azithromycin + gentamicin **OR** gentamicin + doxycycline **OR** Ertapenem
- **Chlamydia:** Azithromycin (1 dose, preferred in pregnancy) **OR** doxycycline (7 days) **OR** amoxicillin (7 days, preferred in pregnancy)
- **Lymphogranuloma Venereum (LGV):** Doxycycline (21 days)
 - **Alternatives:** Azithromycin weekly x 3 weeks
- **Syphilis:** CSF to rule out neurosyphilis
 - **Early disease:** Benzathine penicillin G (1 dose, IM)
 - **Alternatives:** Doxycycline (14 days) **OR** Ceftriaxone (10 days)
 - **Late disease:** Benzathine penicillin G (3 doses weekly, IM)
 - **Alternatives:** Doxycycline (28 days) **OR** Ceftriaxone (10 days, IV/IM) **OR** penicillin desensitization (if other alternatives failed and true penicillin allergy)
- **Human Papillomavirus (HPV):** vaccine (9-26 years old)
 - **Patient administers:** Sinecatchins 10% **OR** Imiquimod 3.75% or 5%
 - **Doctor administers:** Cryotherapy
- **Chancroid:** Azithromycin **OR** Ceftriaxone (both 1 dose)
 - **Alternatives:** Ciprofloxacin (3 days)
- **Pelvic Inflammatory Disease (PID):**
 - **Outpatient:** Ceftriaxone + Doxycycline +/- Metronidazole **OR** Levofloxacin +/- Metronidazole
 - **Inpatient:** Cefoxitin + Doxycycline **OR** Clindamycin + Gentamicin
- **Pubic lice/scabies:** Permethrin 1% (lice), 5% (scabies) **OR** Mixed Pyrethrins (lice)
 - **Alternatives – Scabies:** Sulphur **OR** Ivermectin

Monitoring Parameters

- **Test-of-Cure (TOC)** required for the following:
 - **Syphilis**
 - **Gonorrhea:** if using an alternative regimen, not required for ceftriaxone
 - **PID:** if confirmed gonorrhea or chlamydia

Additional Key Points

- Azithromycin is always added when treating for gonorrhea as it is usually associated with chlamydia (asymptomatic)
- For scabies, all members of household must be treated
- Avoid tetracycline monotherapy, suboptimal activity alone
- Spermicides DO NOT prevent STI transmission
- Sexual partners within 60 days need to be treated EVEN if asymptomatic (gonorrhea/chlamydia)